

| 2026

**PRACTICAL CASE
MANAGEMENT OF COMMON
STI SYNDROMES
HANDBOOK**



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**SPECIALLY ADAPTED FOR USE IN
PRIMARY HEALTH CARE CENTERS**

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CHECKLIST FOR MEDICAL HISTORY TAKING

a. Demographics

1. National Identification number / medical registration number
2. Name
3. Age
4. Date of Birth
5. Sex at birth
6. Gender identity
7. Telephone no. (home / work / if any)
8. Address (indicate if permanent and current residence; indicate if transient, and include landmarks / street number)
9. Next of kin / emergency contact (name, relationship, phone number)
10. Marital status (single / married / separated / divorced / widowed; include duration if applicable)
11. Living arrangements (lives alone / with family / partner / other)
12. Occupation (state type of work, employment status, duration, and if unemployed, reason)
13. Date first seen

b. Present Medical History

1. Reason for present visit
2. Symptoms (state duration / severity / course)
3. Treatment taken (state result / sample drug)

c. Gynaecologic/Obstetric History

1. **Menstrual history:** Age at menarche, cycle pattern, duration, flow, LMP, and recent changes to pattern and associated symptoms
2. **Gynaecologic history:** Last pap smear, past pelvic conditions, gynaecologic

surgeries, infertility, or abnormal bleeding/discharge, douching practices.

3. **Obstetric history:** Gravidity, parity, outcomes of pregnancies, and complications.

d. Past Medical History

1. Chronic illnesses or immunocompromising conditions (indicate if any steroid use)
2. Surgical History (indicate if any genito-anal surgeries)
3. Medication history and drug allergy (state recent antibiotic use within the last 2 weeks)
4. Vaccination History – HBV, HPV (state type and dose completion)
5. Contraception (state the type and frequency of use)
6. History of substance abuse
7. History of blood or blood product transfusions

e. Family History

1. Significant familial conditions (indicate congenital infections, reproductive disorders, infertility)
2. Health status of parents & siblings (include cause of death, if applicable)

CHECKLIST FOR SEXUAL HISTORY TAKING

a. Presenting Complaint

1. Symptoms (discharge, ulcers, rash, itching, pain, dysuria, etc.)
2. Duration and recurrence
3. Recent treatment or self-medication

b. Sexual Exposure History (last 3 months)

1. Currently sexually active
2. Date(s) of last sexual exposure
3. Relationship to last sexual partner(s) — spouse, new / regular partner, or other
4. Sexual practices during last exposure (penetrative or non-penetrative — fisting, fingering, nudging, dipping, masturbation -alone or shared)
5. Anatomical site(s) of exposure

c. Sexual Exposure History (greater than 3 months)

1. Age at sexual debut
2. Number of sexual partners in the past 3 months
3. Number of lifetime sexual partners
4. Type(s) of sexual contact — vaginal, anal, oral, other (specify)
5. Sex of partners — male, female, both
6. Vaginal intercourse during menses
7. Nature of relationships — regular, casual, transactional, anonymous
8. PrEP (use and type)

d. Condom and Contraceptive Use

1. Frequency of condom use (always / sometimes / never)
2. Correct use (start to finish of act)
3. Condom breakage or slippage
4. Condom use with whom (new / regular, casual partners)

5. Use of other contraceptive methods (type / consistency)

e. Partner(s) Sexual History

1. Partner(s) with STI symptoms or diagnosis
2. Contact with new /regular partner(s) in the last 3 months
3. Sex of partners - male, female, both
4. Known HIV positive
5. Partner(s) on PrEP or ART
6. Aware of STI infection
7. History of substance abuse

f. Other Personal Risk Factors:

1. HIV infection
2. History of GBV/IPV

Table 1: Coding for STIs and Other Conditions

New Cases of Syphilis	
Primary	A1
Secondary	A2
Early latent (in the first year of infection)	A3
Cardiovascular	A4
Neurological	A5
All other late and late latent stages	A6
Congenital (aged under 1 year)	A7
Congenital (aged 1 year and over)	A8
New cases of gonorrhoea	
Post-pubertal infection	B1
Pre-pubertal infection	B2
Ophthalmia Neonatorum	B3
New Cases of other STIs	
Chancroid	C1
Lymphogranuloma venereum	C2
Granuloma inguinale	C3
Non-gonococcal urethritis/cervicitis	C4
Non-specific genital infection with arthritis	C5

New Cases of Other STIs continued	
Trichomoniasis	C6
Candidiasis	C7
Herpes simplex virus	C10
Warts (condylomata acuminata/human papilloma virus)	C11
Molluscum contagiosum	C12
HIV infection	C13
Hepatitis B virus	C14
HTLV-1	C15
New cases of other conditions	
Other treponemal disease e.g., yaws	D1
Other conditions requiring treatment in the centre e.g., MPox	D2
Other conditions not requiring treatment in the centre	D3

Table 2: Common Signs and Symptoms of STIs in Men

Symptoms (Reported by Patient)	Signs (Observed on Examination)
Urethral discharge, leaking, or dripping from penis	Urethral discharge (mucoid, purulent, or blood-stained)
Dysuria (burning sensation or pain on urination)	Erythema or swelling of the urethral meatus
Sores or “water bumps” on genitals	Genital ulcers or vesicles
Lower abdominal or pelvic discomfort	Lower abdominal tenderness
Scrotal pain or heaviness	Scrotal swelling or tenderness
Lump, bump, or growth on genitals	Genital warts
Rash	Rash on palms, soles, or body
Rectal pain, discharge, or bleeding	Perianal or rectal discharge or lesions
Swollen glands	Inguinal lymphadenopathy
Sore throat or sores in the mouth	Pharyngeal exudates or ulcers to the oral mucosa

Table 3: Common Signs and Symptoms of STIs in Women

Symptoms (Reported by Patient)	Signs (Observed on Examination)
Vaginal discharge (change in colour, odour, or amount)	Abnormal vaginal discharge (colour, odour, consistency)
Itching or irritation of the vulva or vagina	Vulval or vaginal erythema and edema
Sores or “water bumps” on genitals	Ulcers or vesicles on the genitalia
Dyspareunia (pain during intercourse), lower abdominal or pelvic pain	Cervical discharge or friability, cervical motion tenderness +/- adnexal tenderness or mass
Dysuria (burning or painful urination)	Erythema or swelling of the urethral meatus
Lump, bump, or growth on genitals	Genital warts
Rash	Rash involving palms, soles, or body
Vaginal odour (fishy, foul, or musty smell)	Abnormal vaginal discharge (colour, odour, consistency)
Abnormal vaginal bleeding or spotting	Cervical friability, cervical mass
Swollen glands	Inguinal lymphadenopathy
Painful defecation, rectal discomfort, or discharge	Perianal, rectal discharge, or lesions
Sore throat or sores in the mouth	Pharyngeal exudates or ulcers to the oral mucosa

Figure 1: Algorithm for STI Screening

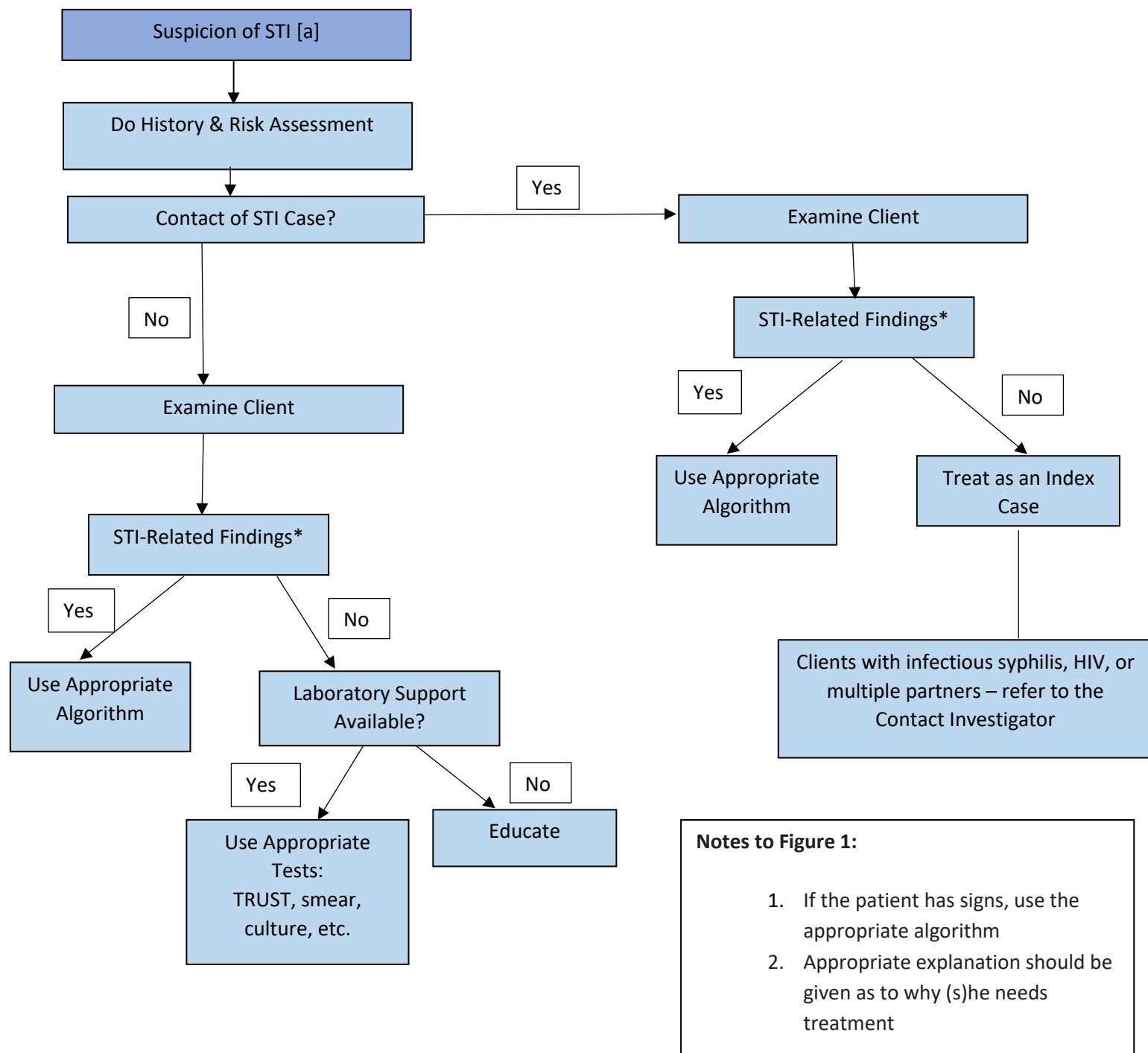
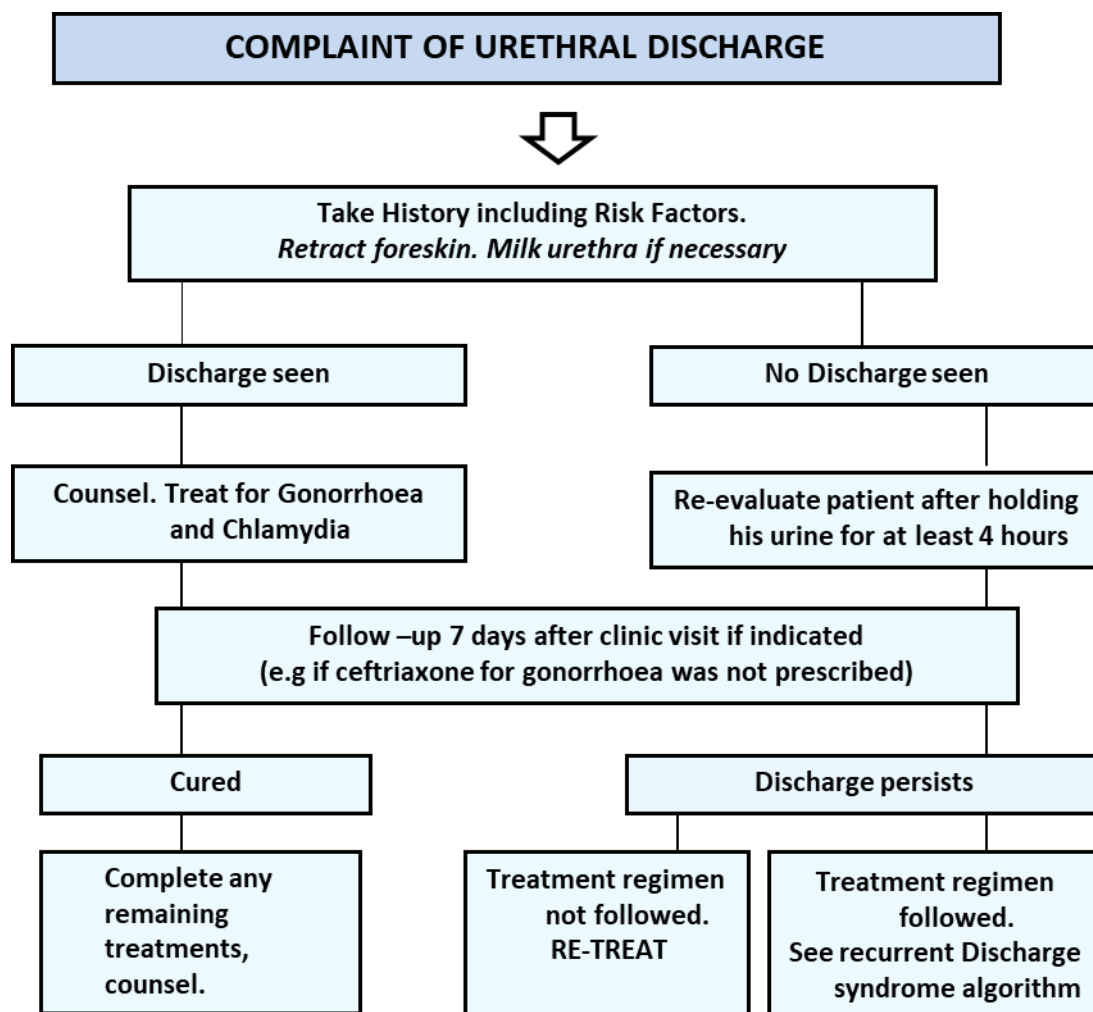


Figure 2: Algorithm for Management of Urethral Discharge



Notes for Figure 2:

- Early morning bead of pus; urinary “threads” in first urine passed; WBCs > 4/HPF on Gram stained smear - suggests chlamydial/non-gonococcal urethritis
- Refer to urologist or hospital, especially patients > 35 years: non-STI causes of urethritis may be present

Figure 3: Algorithm for Management of Persistent Urethral Discharge

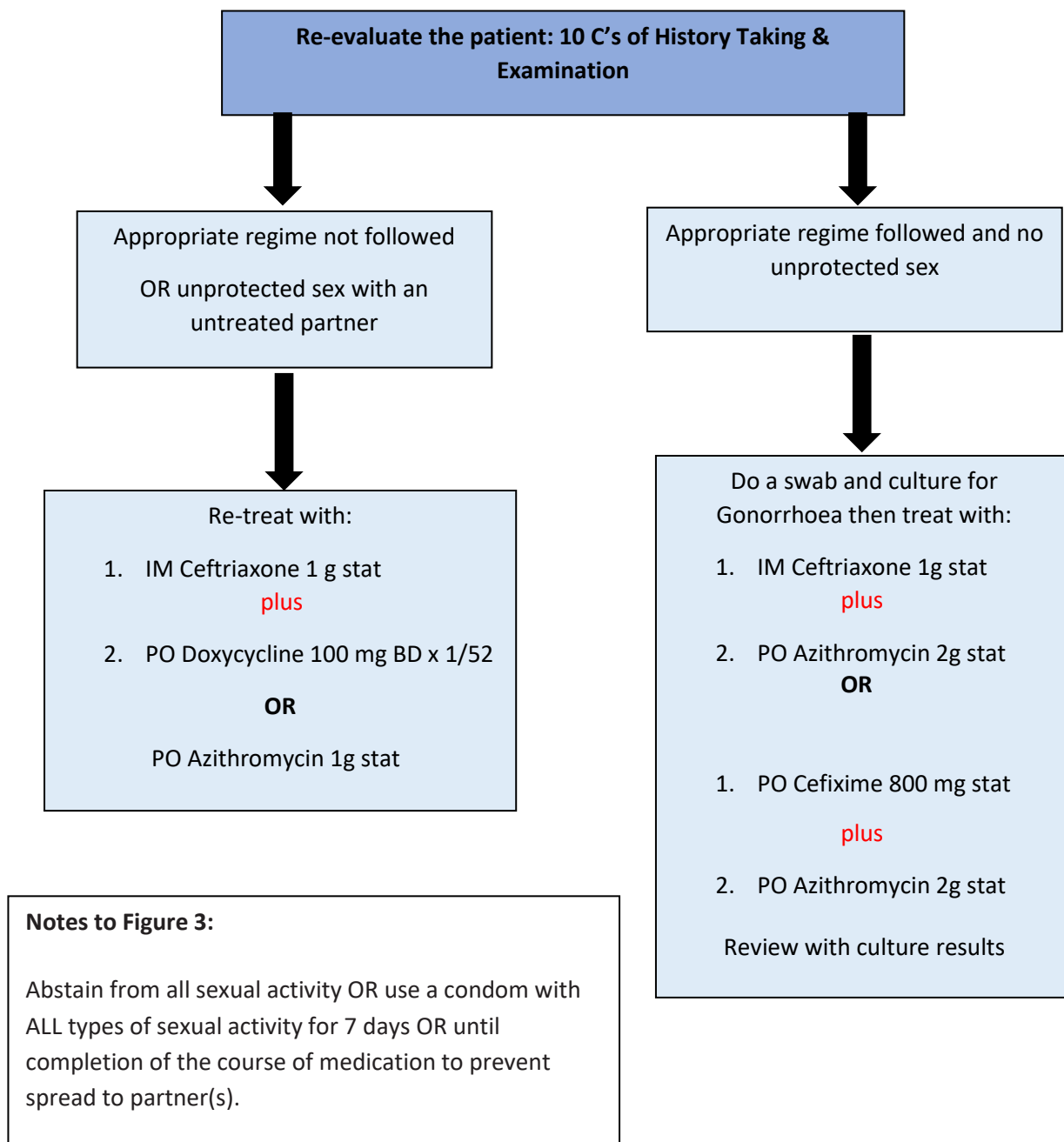


Table 4: Treatment of Urethral Discharge (Gonococcal and Chlamydial Urethritis)

Recommended treatment: Ceftriaxone 1g, intramuscularly, single dose PLUS Doxycycline 100 mg, orally, twice daily for 7 days	<u>Effective substitutes for Ceftriaxone include:</u> Cefixime 800mg, orally, single dose* OR Ciprofloxacin 500mg, orally, single dose	Avoid doxycycline in pregnancy, lactation, and persons 16 years and under.
	<u>Effective substitutes for Doxycycline include:</u> Azithromycin 1g single dose orally OR Erythromycin 500 mg, orally, 4 times a day for 7 days OR Ofloxacin 200–400 mg, orally, twice a day for 7 days	
If symptoms persist or reoccur after 7 days (without evidence of reinfection) 1. Do a swab and test for gonorrhoea and chlamydia (e.g., Nucleic Acid Amplification (NAAT), Culture and Sensitivity tests) 2. Re-treat with: Ceftriaxone 1g, intramuscularly, single dose PLUS Azithromycin 2g, orally PLUS Metronidazole 2g, orally, single dose (OR 500mg, orally, twice daily for 7 days)		Azithromycin may cause gastrointestinal side effects – consider giving Azithromycin 1g at 12-hour intervals.

*If using the alternative option for gonococcal infection (**cefixime 800mg**), add azithromycin 2g to cover chlamydial infection.

Figure 4: Algorithm for Management of Testicular Pain

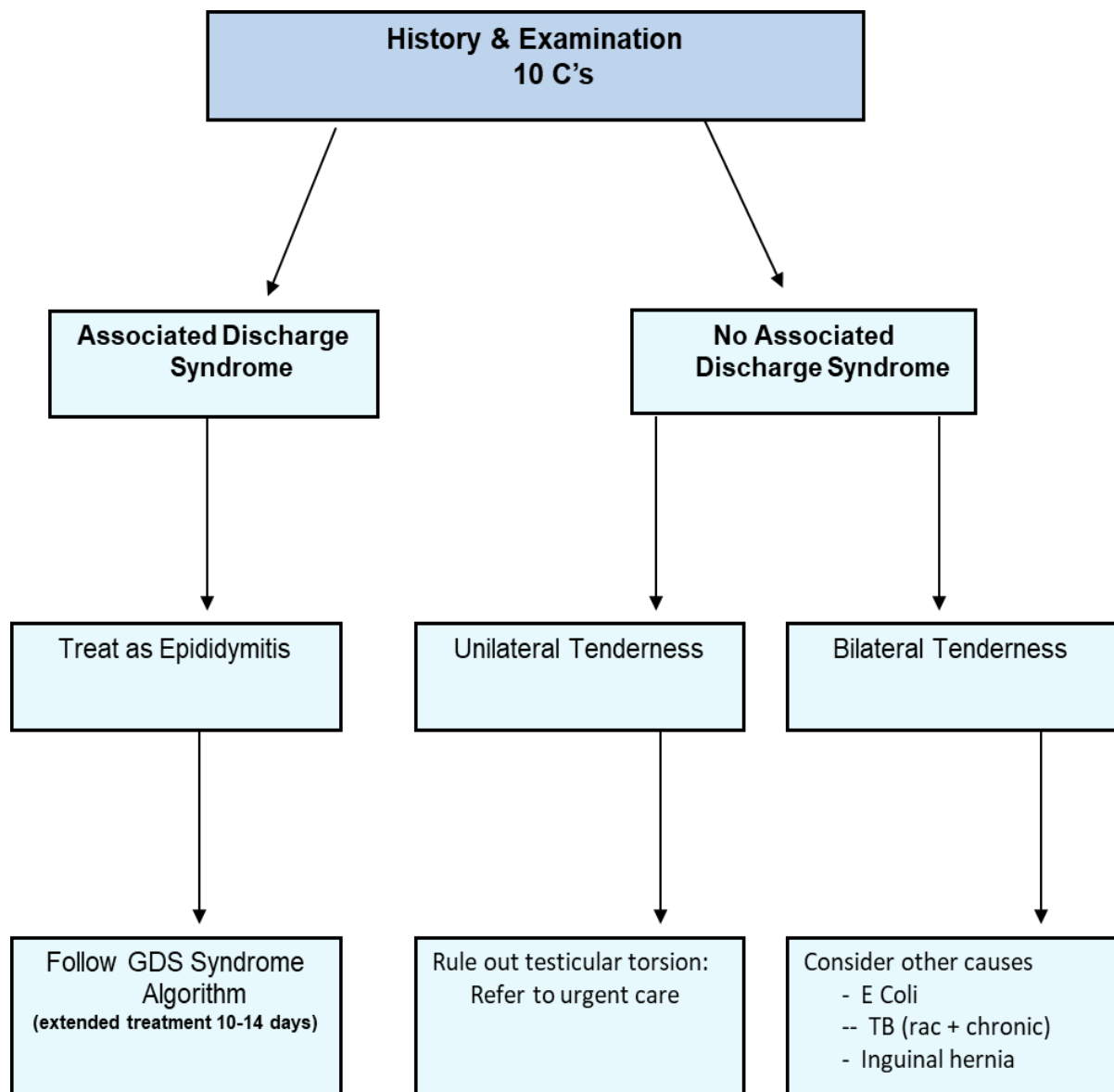
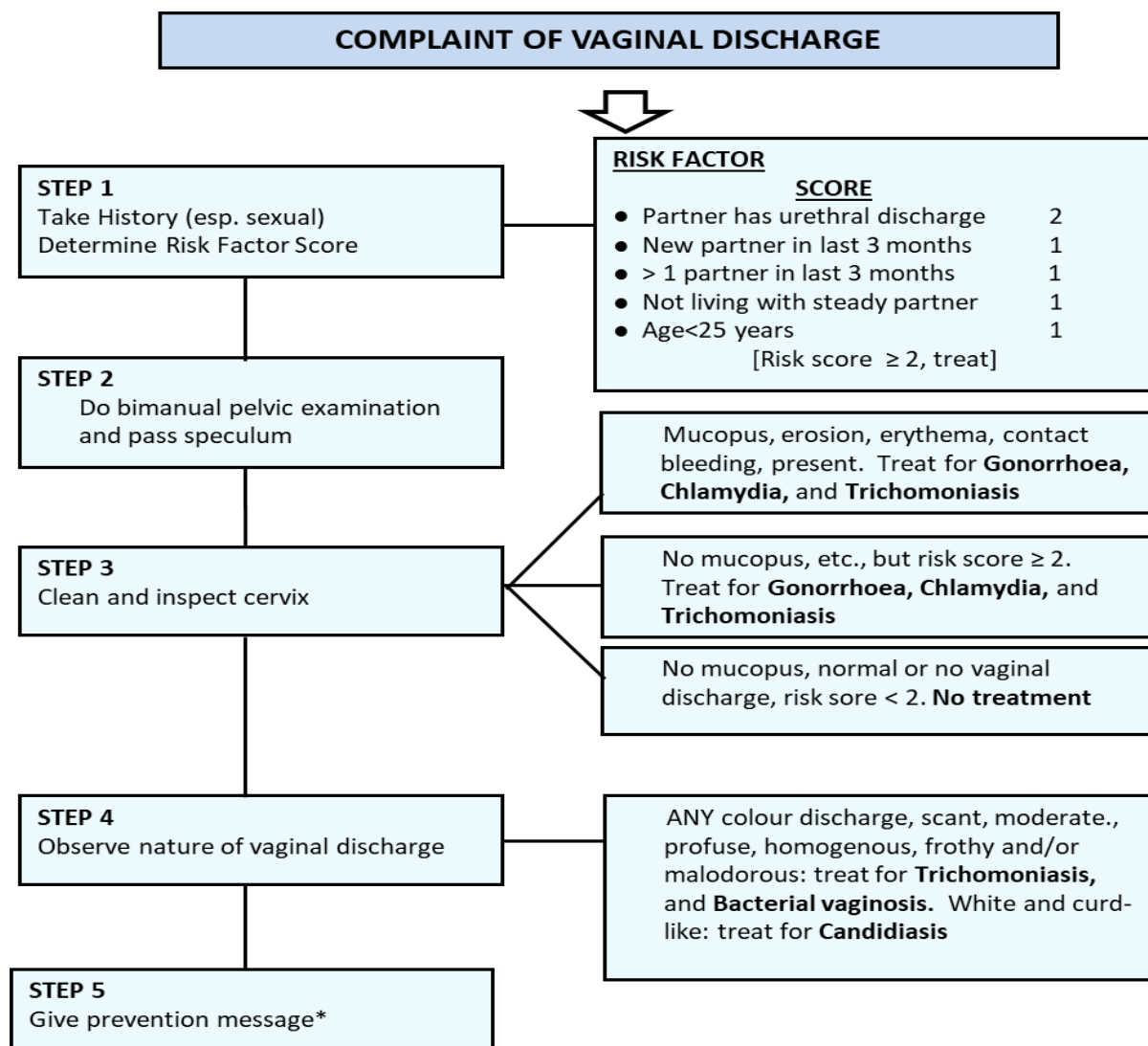


Figure 5: Algorithm for Management of Cervico-vaginal Discharge



Notes to Figure 5:

Comply with medication; condoms; contacts, counsel re risk reduction.

Screen all patients for HIV/Syphilis.

Do pap smears and refer persistent cases of discharge to the gynaecologist.

Table 5: Treatment of Cervico-vaginal Discharge (Gonococcal and Chlamydial Urethritis)

Recommended treatment: Ceftriaxone 1g, intramuscularly, single dose PLUS Doxycycline 100 mg, orally, twice daily for 7 days PLUS Metronidazole 500mg, orally, twice daily for 7 days If cervicitis is present, increase the duration of Doxycycline and Metronidazole to 14 days.	<u>Effective substitutes for Ceftriaxone include:</u> Cefixime 800mg, orally, single dose* OR Ciprofloxacin 500mg, orally, single dose <u>Effective substitutes for Doxycycline include:</u> Azithromycin 1g single dose orally OR Erythromycin 500 mg, orally, 4 times a day for 7 days OR Ofloxacin 200–400 mg, orally, twice a day for 7 days	Avoid doxycycline in pregnancy, lactation, and persons 16 years and under.
If symptoms persist or reoccur after 7 days (without evidence of reinfection) 1. Do a swab and test for gonorrhoea and chlamydia (e.g., Nucleic Acid Amplification (NAAT), Culture and Sensitivity tests) 2. Re-treat with: Ceftriaxone 1g, intramuscularly, single dose PLUS Azithromycin 2g, orally PLUS Metronidazole 2g, orally, single dose (OR 500mg, orally, twice daily for 7 days)		Azithromycin may cause gastrointestinal side effects – consider giving Azithromycin 1g at 12-hour intervals.

*If using the alternative option for gonococcal infection (**cefixime 800mg**) add azithromycin 2g to cover chlamydial infection.

Figure 6: Algorithm for Management of Persistent Vaginal Discharge

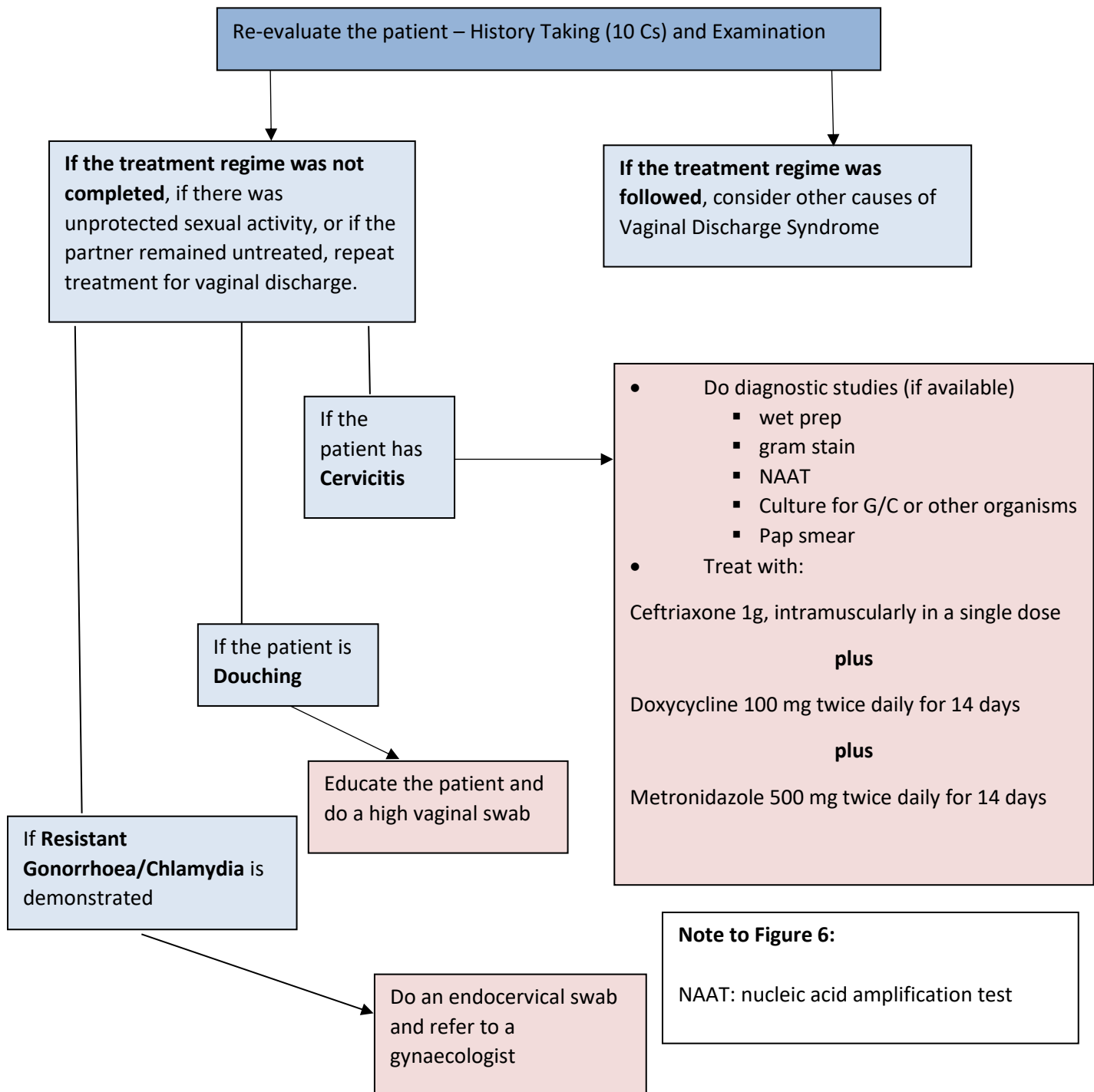


Table 6: Treatment of Vaginal Discharge Syndrome

Aetiology	Recommended Treatment	Effective Substitute	Notes
Bacterial Vaginosis	Metronidazole 400 mg or 500 mg, orally, twice daily for 7 days	• Clindamycin 300 mg, orally, twice daily for 7 days or • Metronidazole 2 grams, orally, single dose or • Metronidazole 200 mg or 250 mg, orally, 3 times a day for 7 days or • Metronidazole gel 0.75%, one full applicator (5 grams) intravaginally, twice a day for 7 days or • Clindamycin 300 mg, orally, twice daily for 7 days	Metronidazole should be avoided during the first trimester of pregnancy unless the potential benefits outweigh the associated risks.
Trichomonas	• Metronidazole 2 grams in a single dose or • Metronidazole 400 mg or 500mg orally, twice daily for 7 days	Effective substitutes for Metronidazole to treat Trichomonas include: • Tinidazole 2 grams orally in a single dose or • Tinidazole 500 mg orally, twice daily for 5 days or Other substitutes: • Metronidazole 200 mg or 250 mg, orally, 3 times a day for 7 days or • Metronidazole gel 0.75%, one full applicator (5 grams) intravaginally, twice a day for 7 days	
Candidiasis	• Clotrimazole vaginal tablet, 100 mg, inserted at night for 7 nights or • Miconazole vaginal pessaries, 200 mg inserted at night for 3 nights	Effective substitutes include: • Fluconazole 150 mg (or 200mg), orally, single dose or • Nystatin, 200,000-unit vaginal tablet, inserted at night for 7 nights Additional substitutes include: • Miconazole 200 mg vaginal pessaries inserted once daily for 3 days or • Clotrimazole vaginal tablet 100 mg inserted at night for 7 days or • Nystatin pessaries 200,000 units, inserted at night for 7 night	

Figure 7: Algorithm for Management of Lower Abdominal Pain/PID

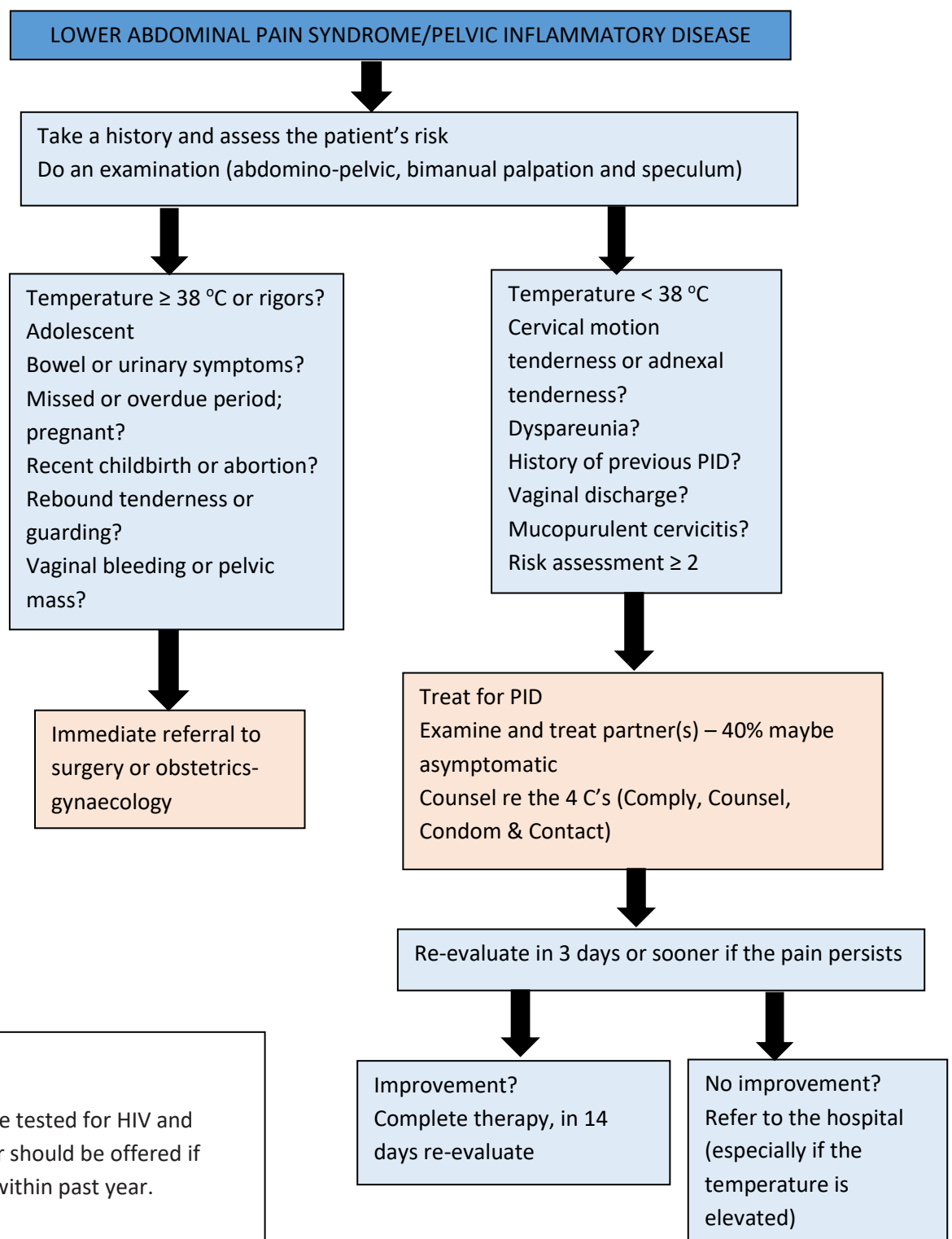


Figure 8: Algorithm for Management of Anal Discharge

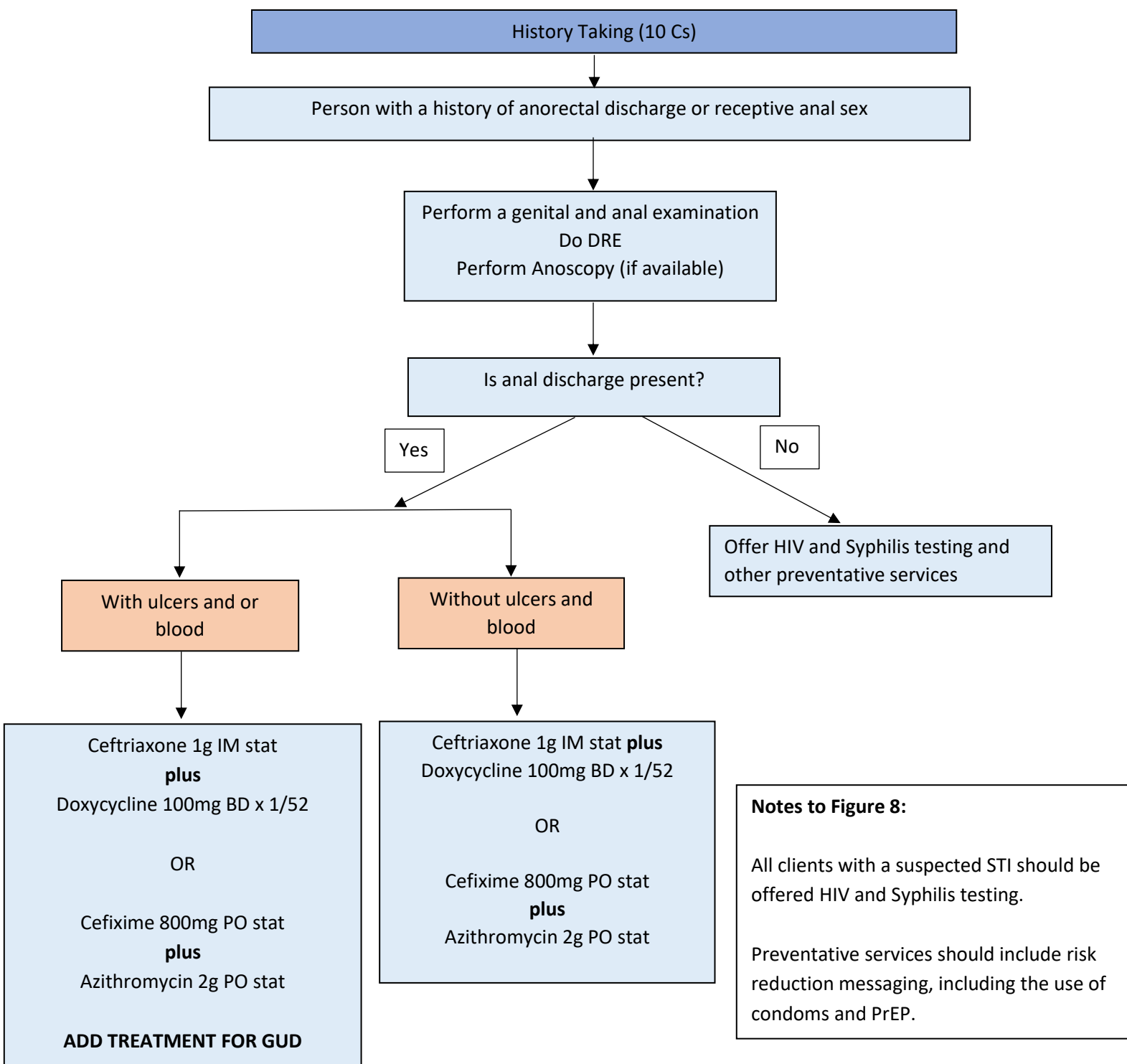


Table 7: Treatment of Anal Discharge Syndrome

Recommended Treatment	Effective Substitutes	Notes
Ceftriaxone 1g, intramuscularly, single dose PLUS Doxycycline 100 mg, orally, twice daily for 7 days	Cefixime 800mg, orally, single dose PLUS Azithromycin 2g, orally	Azithromycin may cause gastrointestinal side effects – consider giving Azithromycin 1g at 12-hour intervals.

Figure 9: Algorithm for Management of Genital Ulcer Disease (GUD)

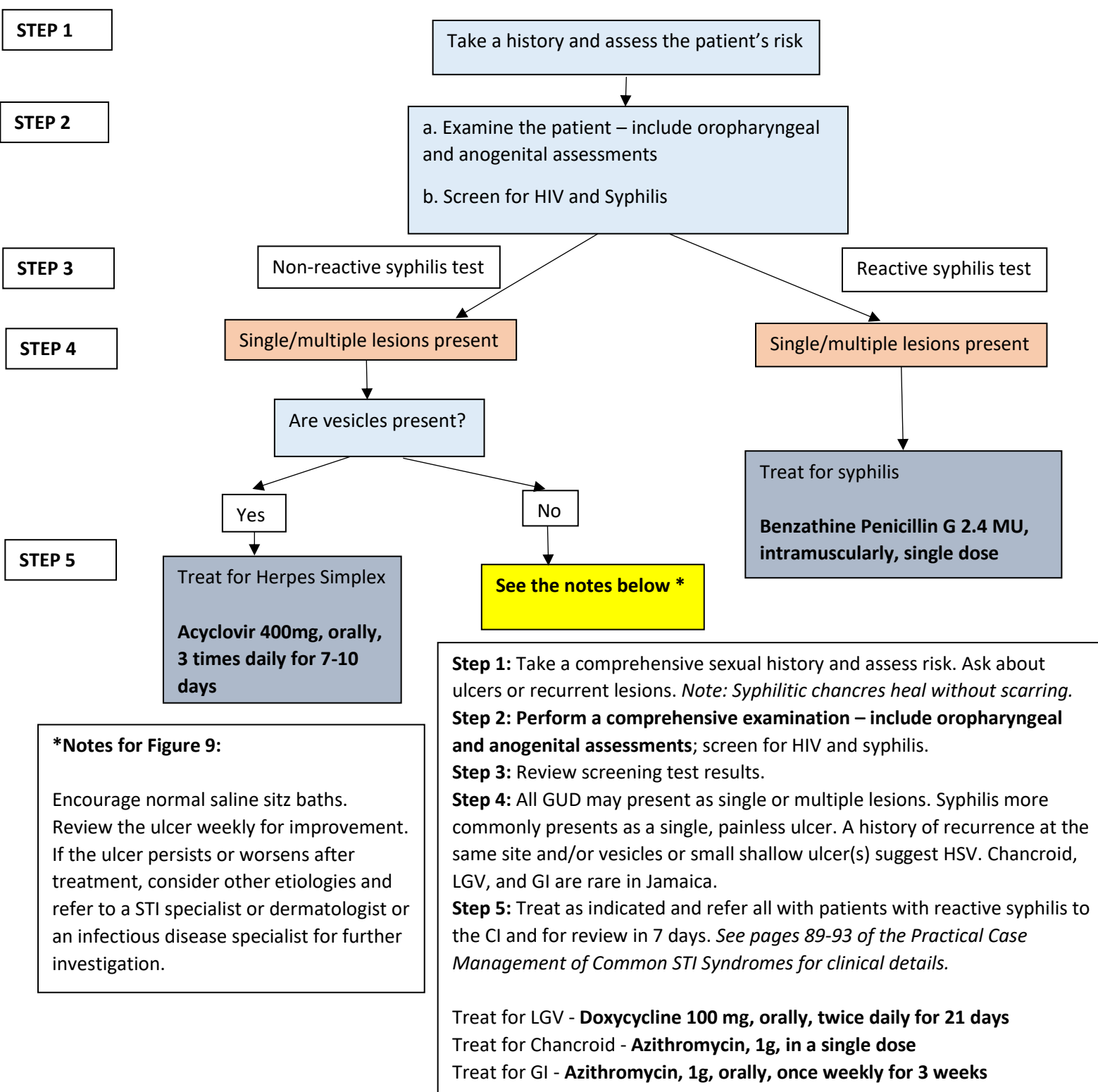


Figure 10: Field Algorithm for Laboratory Diagnostic Testing for Syphilis

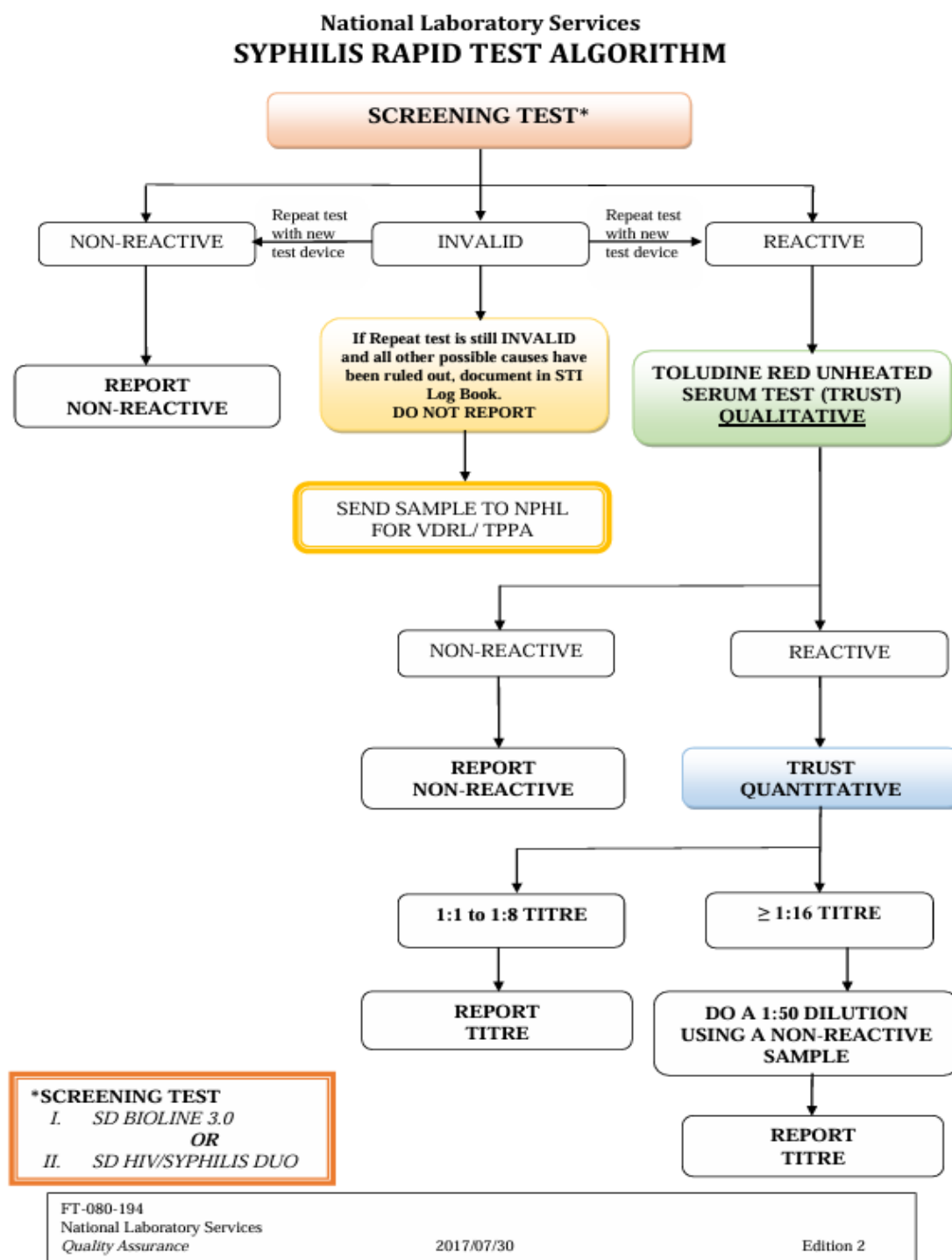


Table 8: Syphilis Stages and Test Results

Stage of Syphilis	Treponemal Test Result e.g. SD Bioline, TPPA*, POC kits	Non-Treponemal Test Result e.g. VDRL, TRUST
Primary (sore) (Incubation period: 9-90 days)	Reactive (78 -86%)	Reactive (76-84%), > R1:8
Secondary (rash)(usually occurs 2-8 weeks after primary stage)	Reactive (100%)	Reactive (100%), > R1:8
Early Latent (infection of less than 1 year duration without clinical signs)	Reactive (97 – 100%)	Reactive (95-100%), > R1:8
Late Latent (infection of 1 or more years duration without clinical signs)	Reactive (100%)	Reactive, > R1:8
Tertiary	Reactive (94 – 96%)	Reactive (71-73%), > R1:8

Notes to table 8:

*SD Bioline

TPPA - Treponema pallidum particle agglutination assay.

POC - Point-of-care

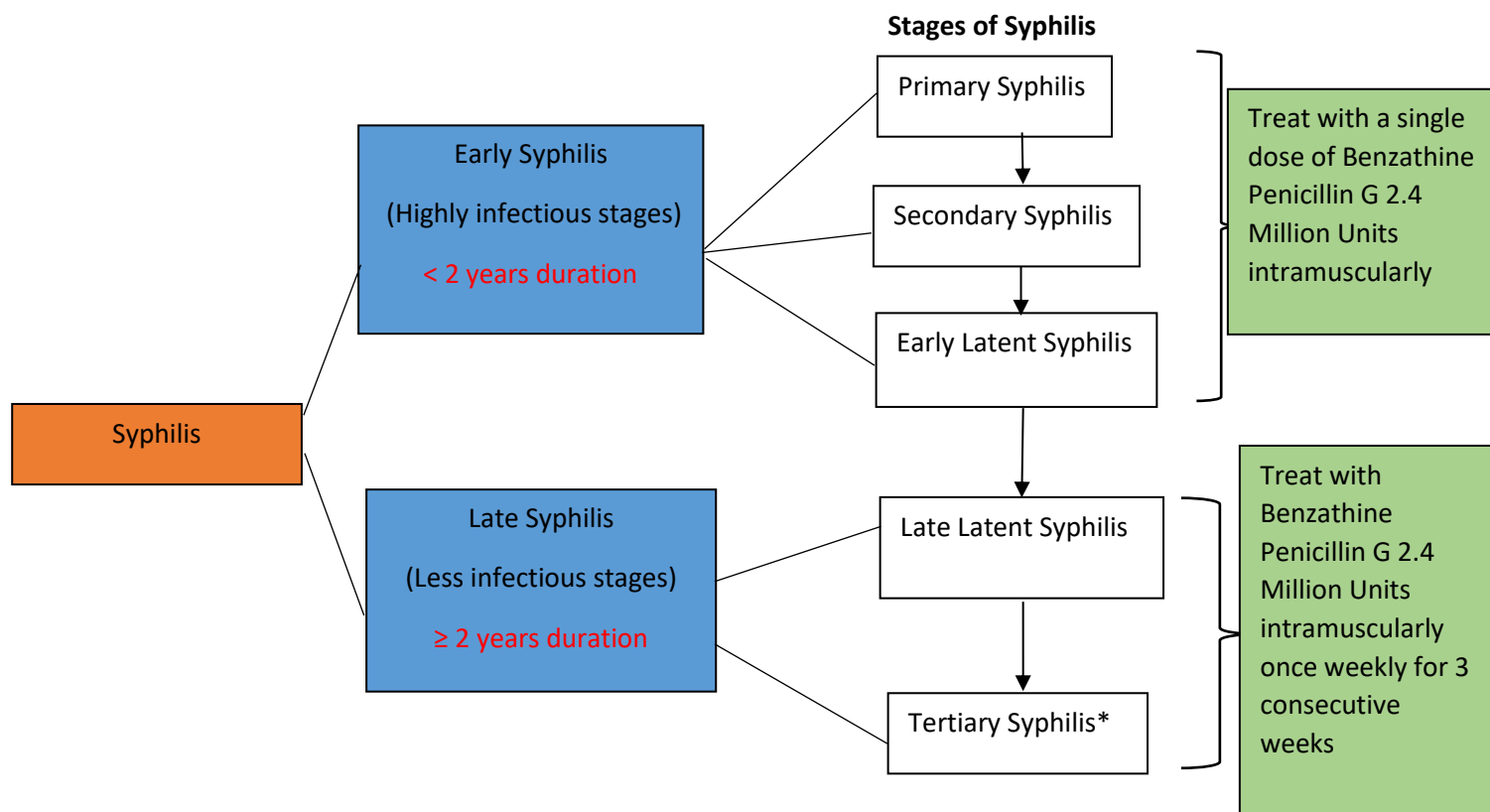
VDRL - Venereal disease research laboratory

TRUST - Toluidine red unheated serum test

Table 9: Guide to the Interpretation of Syphilis Serologic Test Results

Treponemal test result e.g. SD Bioline, TPPA*, POC kits	Non-Treponemal test result e.g. VDRL, TRUST	Interpretation
Reactive	Reactive	Syphilis may be treated or untreated. If the initial titre is $\geq$ 1:8, it is likely indicative of infectious syphilis. However, if the pre-treatment titre was high and there is a documented fourfold decrease in titre following treatment, the patient is likely no longer infectious.
Reactive	Non-reactive	This pattern is typically indicative of treated syphilis but may also suggest primary syphilis, very early latent syphilis, or late latent syphilis. Differentiation relies on a detailed history and clinical examination.
Non-reactive	Reactive	Biological false positive

Figure 11: Algorithm for the Management of Syphilis



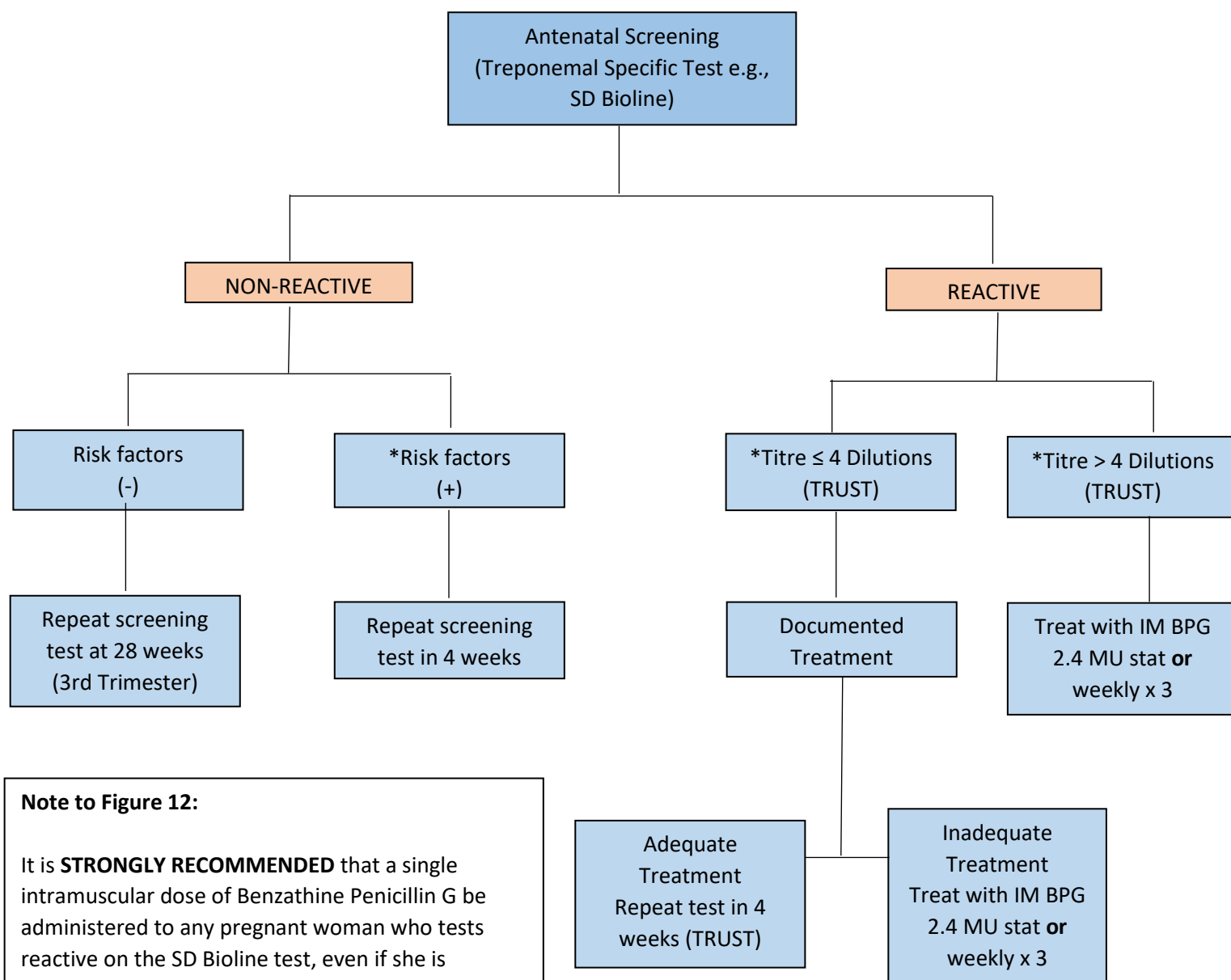
Note to Figure 11:

*First-line treatment for tertiary syphilis is recommended for cases without neurological or ocular involvement.

Table 10: Treatment of Genital Ulcer Disease - Syphilis

Aetiology	Treatment
Syphilis (<i>Primary, secondary, and early latent stages</i>: these stages are considered highly infectious and occur within two years of infection.)	• Benzathine Penicillin G (BPG) 2.4 million units, intramuscularly in a single dose OR • Doxycycline 100 mg, orally, twice daily for 14 days OR • Erythromycin 500 mg, orally, four times daily for 14 days
Syphilis (<i>Late latent and tertiary stages</i>: untreated individuals in these stages are less likely to transmit the infection as latency progresses and occurs more than two years of infection.)	• Benzathine Penicillin G (BPG) 2.4 million units, intramuscularly once weekly for three (3) consecutive weeks OR • Doxycycline 100 mg, orally, twice daily for 30 days OR • Erythromycin 500 mg, orally, four times daily for 30 days

Figure 12: Algorithm for Management of Maternal Syphilis



Note to Figure 12:

It is **STRONGLY RECOMMENDED** that a single intramuscular dose of Benzathine Penicillin G be administered to any pregnant woman who tests reactive on the SD Bioline test, even if she is serofast.

*If the history or clinical assessment indicates an active syphilis infection, provide full treatment according to the identified stage and refer the case to the Contact Investigator (CI) for partner management.

All syphilis seropositive mothers allergic to penicillin must be referred to an internist for penicillin desensitization. Medications such as erythromycin do not cross the placental barrier and is not adequate to treat the foetus.

Table 11: Treatment for Maternal Syphilis

Condition	Recommended Treatment
Primary (≤ 2 years)	Benzathine Penicillin G, 2.4 million units, intramuscularly, single dose
Secondary and Early Latent (≤ 2 years)	Benzathine Penicillin G, 2.4 million units, intramuscularly, single dose
Late Latent and Tertiary (> 2 years)	Benzathine Penicillin G, 2.4 million units, intramuscularly, single dose for 3 consecutive doses one week apart (First-line treatment for tertiary syphilis is recommended for cases without neurological or ocular involvement.)
Penicillin-Allergic Patients with Syphilis	Refer to internal medicine or an infectious diseases specialist for penicillin desensitisation.

Figure 13: Algorithm for the Management of Congenital Syphilis

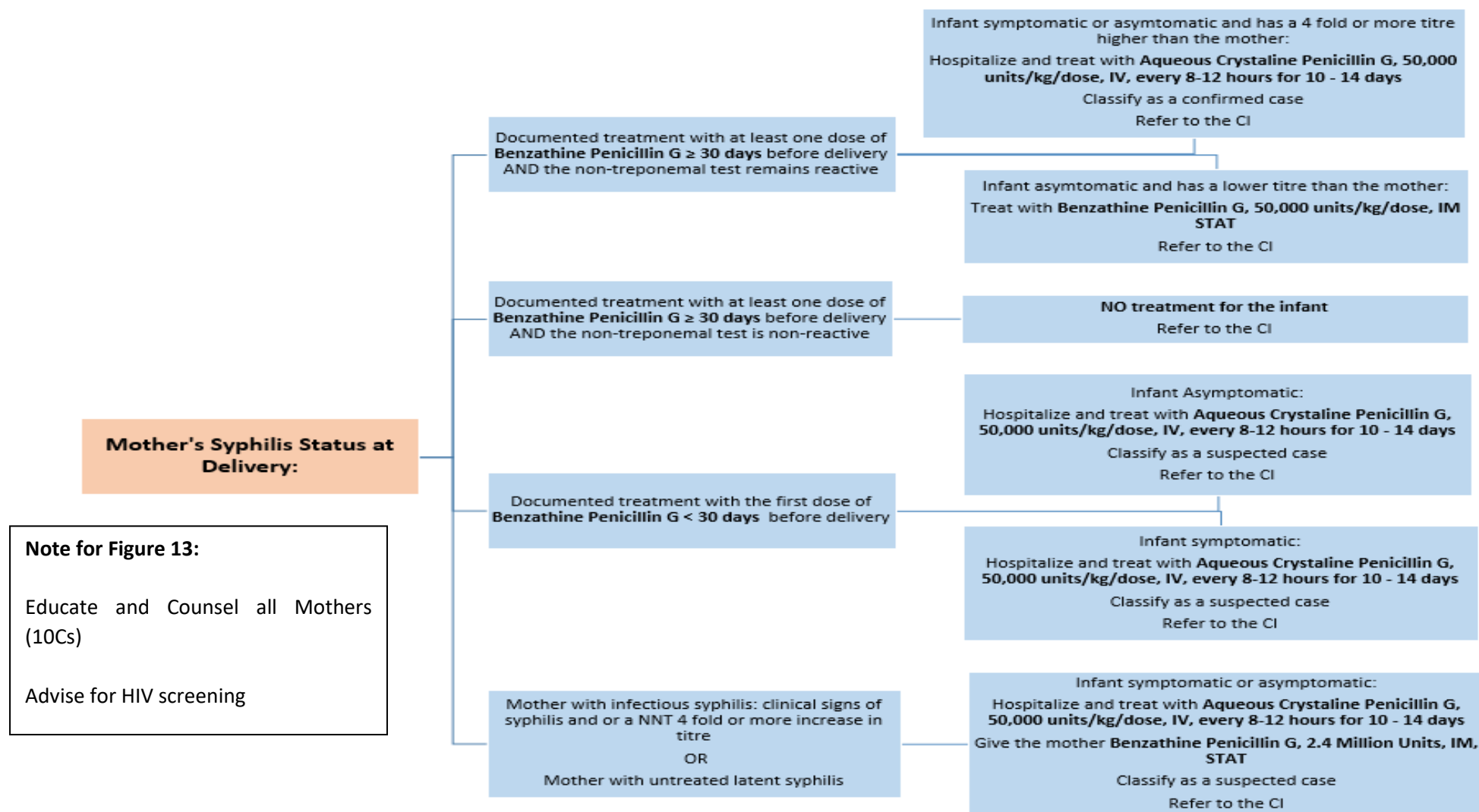


Table 12: Treatment for Congenital Syphilis

Condition	Recommended Treatment
Early Congenital Syphilis < 2 years	Aqueous Crystalline Penicillin G (ACPG): 50, 000 units/kg/dose IV every 8 – 12 hours (100,000-150,000 units daily) for 10-14 days. OR Aqueous Procaine Penicillin G (APPG): 50,000 units/kg IM stat daily for 10-14 days. All syphilis seropositive antenatal women must be treated with at least a single dose of Benzathine Penicillin G, 2.4 Million Units, intramuscularly, for greater than 30 days before delivery. When the mother has been treated with erythromycin during pregnancy or BPG< 30 days before delivery, this is a probable case of congenital syphilis, which requires admission to a secondary care facility for confirmation and appropriate treatment. (N.B. Erythromycin passes the placental barrier poorly).

Table 13: Treatment of Genital Ulcer Disease – Herpes Simplex Virus (HSV)

Aetiology	Treatment
Herpes Simplex Virus	<u>Symptomatic primary episode:</u> • Acyclovir 400 mg, orally, 3 times/day for 7 - 10 days OR • Acyclovir 200 mg, orally, 5 times/day for 7 - 10 days OR • Valacyclovir 500 mg twice daily for 7 - 10 days <u>Recurrent episodes:</u> • Acyclovir 400 mg, orally, 3 times/day for 5 days OR • Acyclovir 800 mg, orally, twice daily for 5 days OR • Acyclovir 800 mg, orally, 3 times/day for 2 days OR • Valacyclovir 500 mg, orally, twice daily for 5 days <u>Suppressive therapy:</u> • Acyclovir 400 mg, orally, twice daily for 6 months then reassess (MANAGEMENT MAY EXTEND UP TO A YEAR) OR • Valacyclovir 500 mg, orally, once daily for 6 months then reassess (MANAGEMENT MAY EXTEND UP TO A YEAR) Plus Saline Sitz Baths as needed

Table 14: Treatment of Genital Ulcer Disease – Chancroid, Granuloma Inguinale and Lymphogranuloma Venereum

Aetiology	Treatment
Chancroid	Azithromycin 1 g, orally, in a single dose OR Ceftriaxone 250 mg, intramuscularly, in a single dose OR Ciprofloxacin 500 mg, orally, twice daily for three (3) days OR Erythromycin 500 mg, orally, 3 times daily for 7 days
Granuloma Inguinale (Treat until completely cured. Follow up monthly for at least 6 months.)	• Azithromycin 1 g, orally, once weekly for 3 weeks and until all lesions have completely healed OR • Azithromycin 500 mg, orally, once weekly for 3 weeks and until all lesions have completely healed. OR • Doxycycline 100 mg, orally, twice daily for 3 weeks and until all lesions have completely healed. OR • Erythromycin 500 mg, orally, 4 times/day for 3 weeks and until all lesions have completely healed. OR • Trimethoprim-sulfamethoxazole (160 mg/800 mg), one double-strength (1DS) tablet, orally, twice daily for 3 weeks and until all lesions have completely healed.
Lymphogranuloma Venereum	• Doxycycline 100 mg, orally, twice daily for 21 days OR • Azithromycin 1 g, orally, once weekly for 3 weeks OR • Erythromycin, 500 mg, orally, 4 times/day for 21 days

Table 15: Prescribing Pre-Exposure Prophylaxis (PrEP)

PrEP Method	How is it taken?	Who is it for?	What is it for?
Daily oral PrEP	One pill is taken daily	All populations	Preventing HIV acquisition during all types of exposures
Event-driven (ED) PrEP	Two pills are taken 2-24 hours before sex and then a pill each following day until two days after the last potential sexual exposure	People assigned male at birth, not using exogenous hormones, <u>who can plan their sexual activity</u>	Preventing HIV acquisition during SEXUAL EXPOSURES ONLY

Notes to table 15:

PrEP is to be prescribed to people who are HIV negative with a substantial risk of acquiring HIV and have a creatinine clearance of > 60 ml/min.

Table 16: Antiretroviral Therapy for Adolescents and Adults living with HIV/AIDS

First Line ARVs		
First choice	Tenofovir/Lamivudine (TDF/3TC)	Dolutegravir (DTG)
Second choice	Tenofovir/Lamivudine (TDF/3TC)	Efavirenz (EFV)
Second Line ARVs		
First choice	Abacavir/Lamivudine (ABC/3TC)	Atazanavir/Ritonavir (ATV/r)
Second choice	Abacavir/Lamivudine (ABC/3TC)	Lopinavir/Ritonavir (LPV/r)

Table 17: Recommended Post-Exposure Prophylaxis (PEP) following Occupational and Non-Occupational Exposure to a Known or Unknown Source

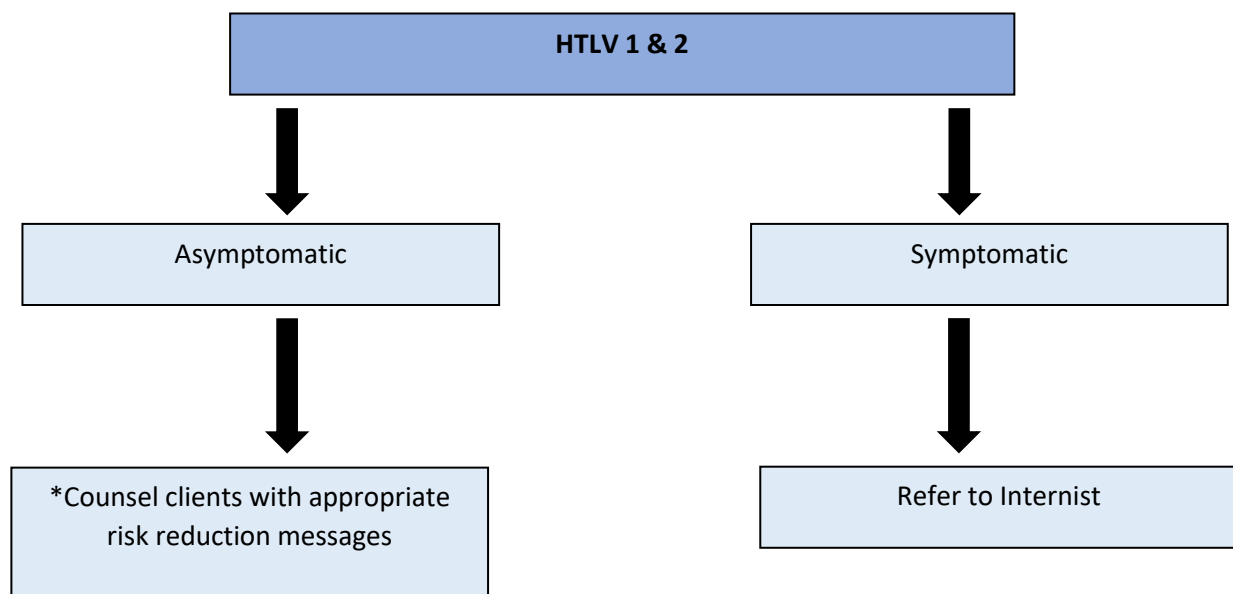
Type of Exposure	Risk	Source	Antiretroviral	Suggested Regimen
Percutaneous	High risk	Known HIV positive	Recommended	Tenofovir/Lamivudine/Dolutegravir (TDF/3TC/DTG) Duration 4 weeks
	Low risk	Unknown Serostatus	Should be offered (Consider for unknown serostatus)	Tenofovir/Lamivudine/Dolutegravir (TDF/3TC/DTG) Duration 4 weeks

Type of Exposure	Risk	Source	Antiretroviral	Suggested Regimen
Mucous Membranes, Non-intact skin	Large volume	Known HIV positive	Should be offered (Consider for unknown serostatus)	Tenofovir/Lamivudine/Dolutegravir (TDF/3TC/DTG) Duration 4 weeks
	Small volume (few drops)	Unknown Serostatus	Should be offered (Not recommended for unknown serostatus)	Tenofovir/Lamivudine/Dolutegravir (TDF/3TC/DTG) Duration 4 weeks

Notes for table 17:

- PEP should be offered ideally within hours of exposure; however, clinical benefit remains up to 72 hours.
- Alternatives for ART choices include:
 - o For Tenofovir/Lamivudine (TDF/3TC), the alternative is Abacavir/Lamivudine (ABC/3TC).
 - o For Dolutegravir (DTG), the alternative is Atazanavir/Ritonavir (ATV/r).
 - o These alternatives, similarly to the recommended option should be given for 4 weeks.

Figure 14: Algorithm for Management of Human T Lymphotropic Virus Type I/II (HTLV-I/II)



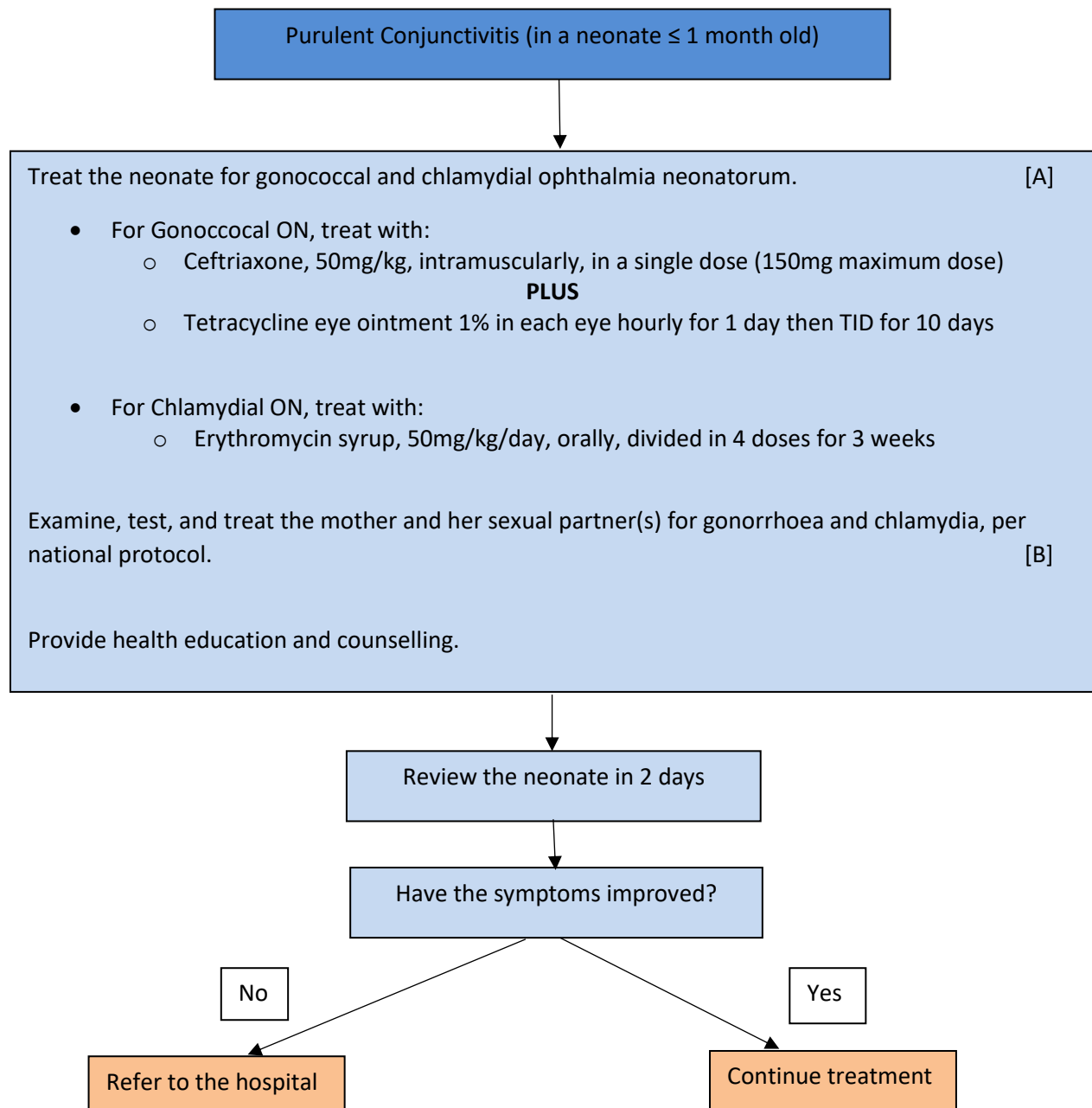
Notes to Figure 14:

Clients who test positive for HTLV I or II should be encouraged to use condoms during sexual activity.

Pregnant women with HTLV should also be encouraged **NOT** to breastfeed.

Figure 15: Algorithm for Management of Ophthalmia Neonatorum (ON)

(Without Laboratory support)



Notes to Figure 15:

ON is a medical emergency and best managed in hospital

[A] Given the intermittent unavailability of erythromycin syrup and its poor activity against *N. gonorrhoeae*, use tetracycline eye ointment plus systemic therapy active against the *N. gonorrhoeae*.

[B] In the absence of a confirmed diagnosis, the decision to notify partner(s) should take into account local cultural and epidemiological factors.



HIV/STI/TB Unit

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